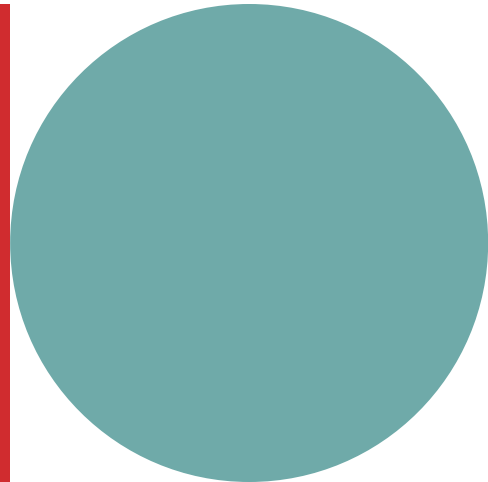


Young patient with oligometastatic gastric cancer

Peter Grell

Masaryk Memorial Cancer Institute, Brno, Czech Republic



Case report – the patient



Patient:

- **Female, 50 years old** (at the time of diagnosis)
- Family history: father rectal cancer, age 80



- **Medical history:**
 - No medical history
 - Medication: PPI, iron substitution



- **May 2023** – back pain, weight loss, 7 kg in 3 months
- **Colonoscopy, June 2023** - negative
- **Gastrosocopy, June 2023** - infiltration in the stomach body and antrum, on a small curvature, continues to the front wall of the stomach antrum

Case report – diagnostics



Histology, June 2023:

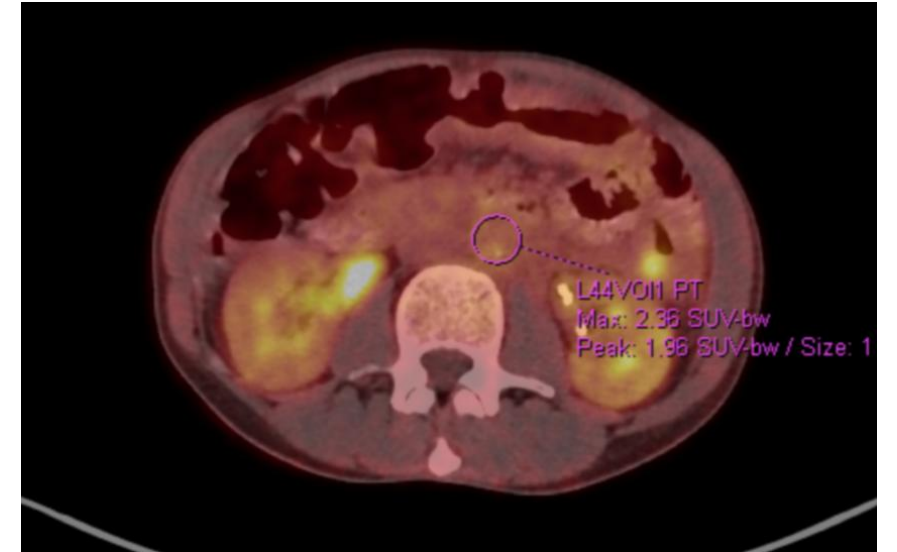
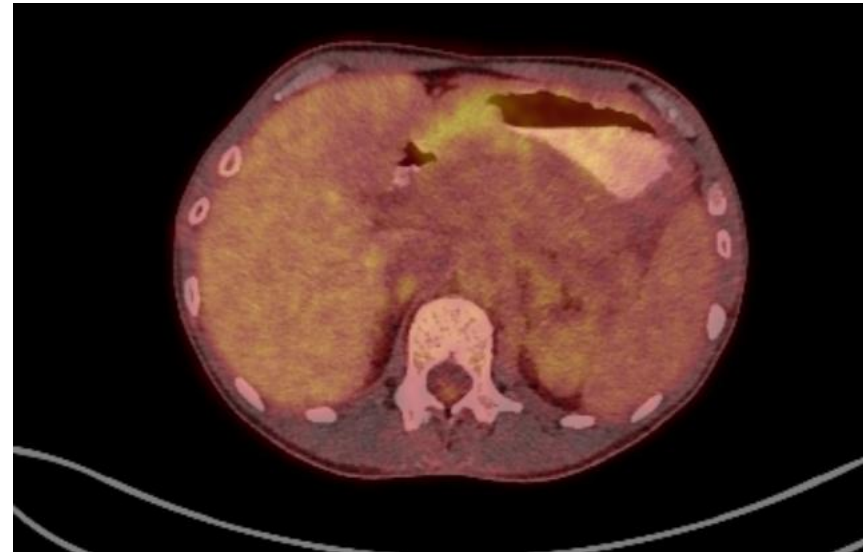
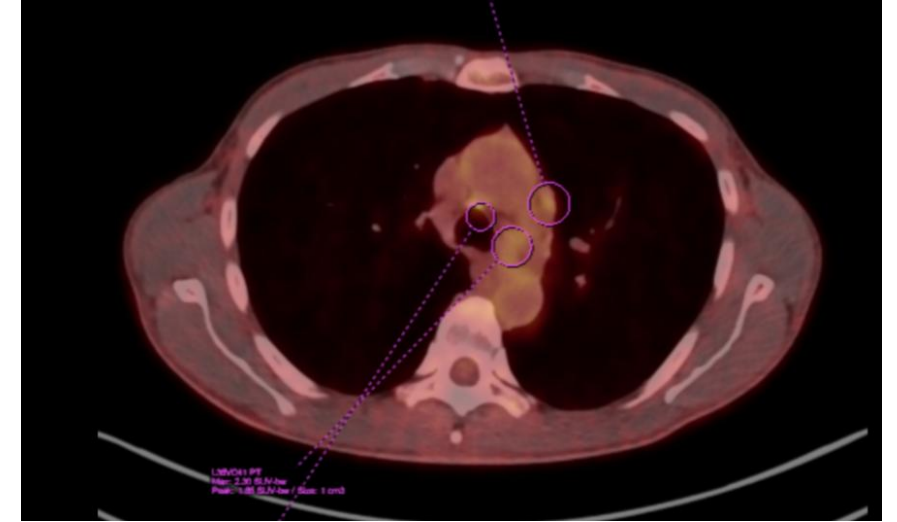
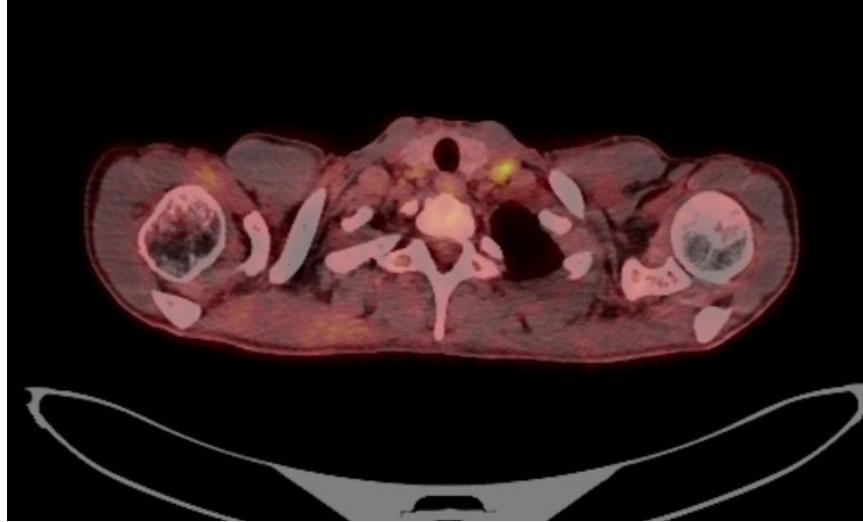
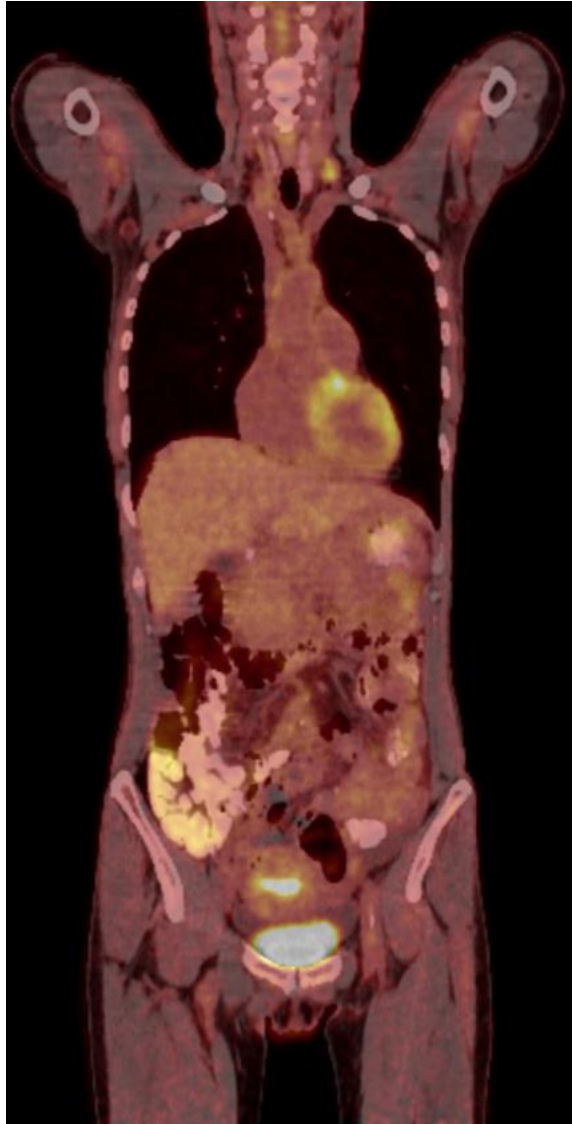
- **poorly differentiated adenocarcinoma of the diffuse type** - low cohesive, of the **signet ring-cell** type in the biopsy of the ulceration



PET/CT, scan July 2023 :

- **diffuse thickening** of the stomach wall
- **moderate FDG accumulation** in the area of the fundus and body of the stomach, pyloric area SUV 3.68
- **Pathological supraclavicular lymph node on the left (12 mm), SUV 3.86**
- Enlarged **mediastinal lymph nodes** (max. 15 mm) with **borderline** FDG accumulation, max SUV 2.74
- **Paraaortal** pathological lymphadenopathy (24 mm), radix mesenterii (15 mm), **borderline** FDG, max SUV 2.5-3.2
- a small amount of **ascites**

Case report – diagnostics



Case report – diagnostics



Histology from pathological supraclavicular lymph node, July 2023

- In the core-cut biopsy a **metastasis of poorly differentiated adenocarcinoma**, a finding well **compatible with gastric origin**

Cytology of ascitic fluid: negative

Molecular testing:

- **HER2 immunohistochemistry 1+ – NEGATIVE**
- **MMR immunohistochemistry – no absence – NEGATIVE**
- **PD-L1 TPS 0%, CPS 10**
- **NTRK immunohistochemistry - NEGATIVE**



Tumor markers:

- Elevated **CA 72-4** 13.8 kU/l (ULN < 6.9), normal level CEA, CA19-9

Diagnosis: T3N1M1(supraclavicular lymph node, the rest (paraaortal, mesenterial) - was evaluated as non – cancer etiology)

Case report – treatment



Treatment:

- Patient was enrolled into the clinical trial:
- **mFOLFOX + nivolumab + Claudin 18.2 ADC**, every 2 weeks
- **1. cycle administered in August 2023**
- (in October 2023 treatment with Claudin 18.2 ADC stopped)
- the treatment was very well tolerated

Case report – treatment efficacy



First restaging, CT scan, October 2023

- **Stationary thickening of gastric wall**
- **Regression of the supraclavicular lymph node (8 mm)**

CT scan, November 2023

- **Stationary thickening of gastric wall**
- **Regression of the supraclavicular lymph node (5 mm)**

PET/CT, February 2024

- **No pathological accumulation in the stomach**
- **Size and metabolic regression of the left supraclavicular lymph node**

PET/CT, August 2024

- **No pathological accumulation in the stomach**
- **Reactive lymphatic mediastinal nodes**

Case report – treatment efficacy



Endoscopy, June 2024:

- **Two small ulcerations on the small curvature** (on the edge of the antrum and body of the stomach)

Endoscopy, September 2024:

- **Two small ulcerations** are present on the small curvature in the antral region



Histology from Endoscopy, September 2024:

- **Adenocarcinoma, diffuse type**



Ultrasound of the supraclavicular lymph node, September 2024

- **two nodes** near the vascular bundle, **physiol. appearance** size 10 and 14 mm, with preserved stratification

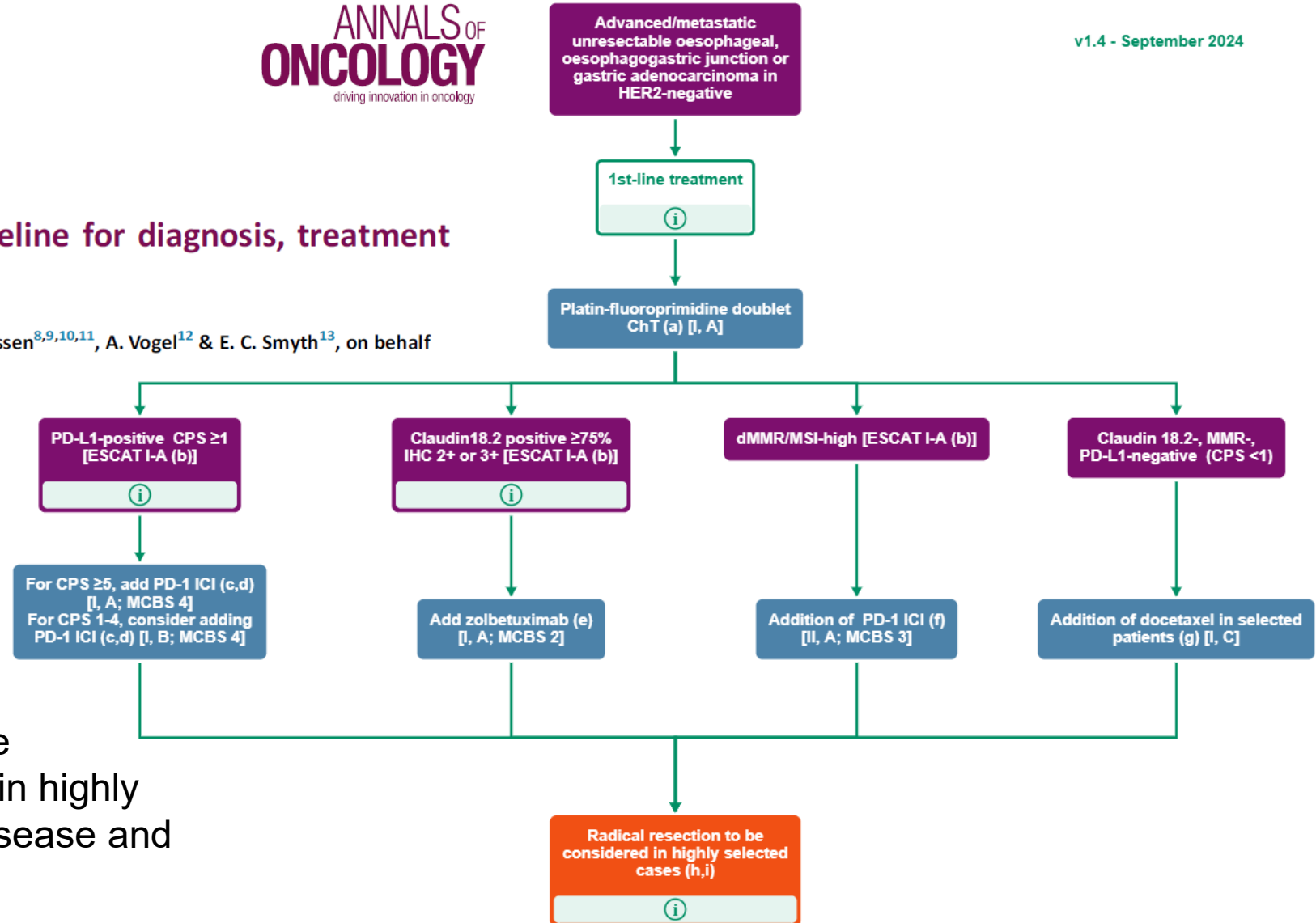
Tumor markers: normal levels of CEA, CA 72-4, CA19-9

Case report – surgery or no surgery

SPECIAL ARTICLE

Gastric cancer: ESMO Clinical Practice Guideline for diagnosis, treatment and follow-up[☆]

F. Lordick¹, F. Carneiro^{2,3,4}, S. Cascinu⁵, T. Fleitas⁶, K. Haustermans⁷, G. Piessen^{8,9,10,11}, A. Vogel¹² & E. C. Smyth¹³, on behalf of the ESMO Guidelines Committee^{*}



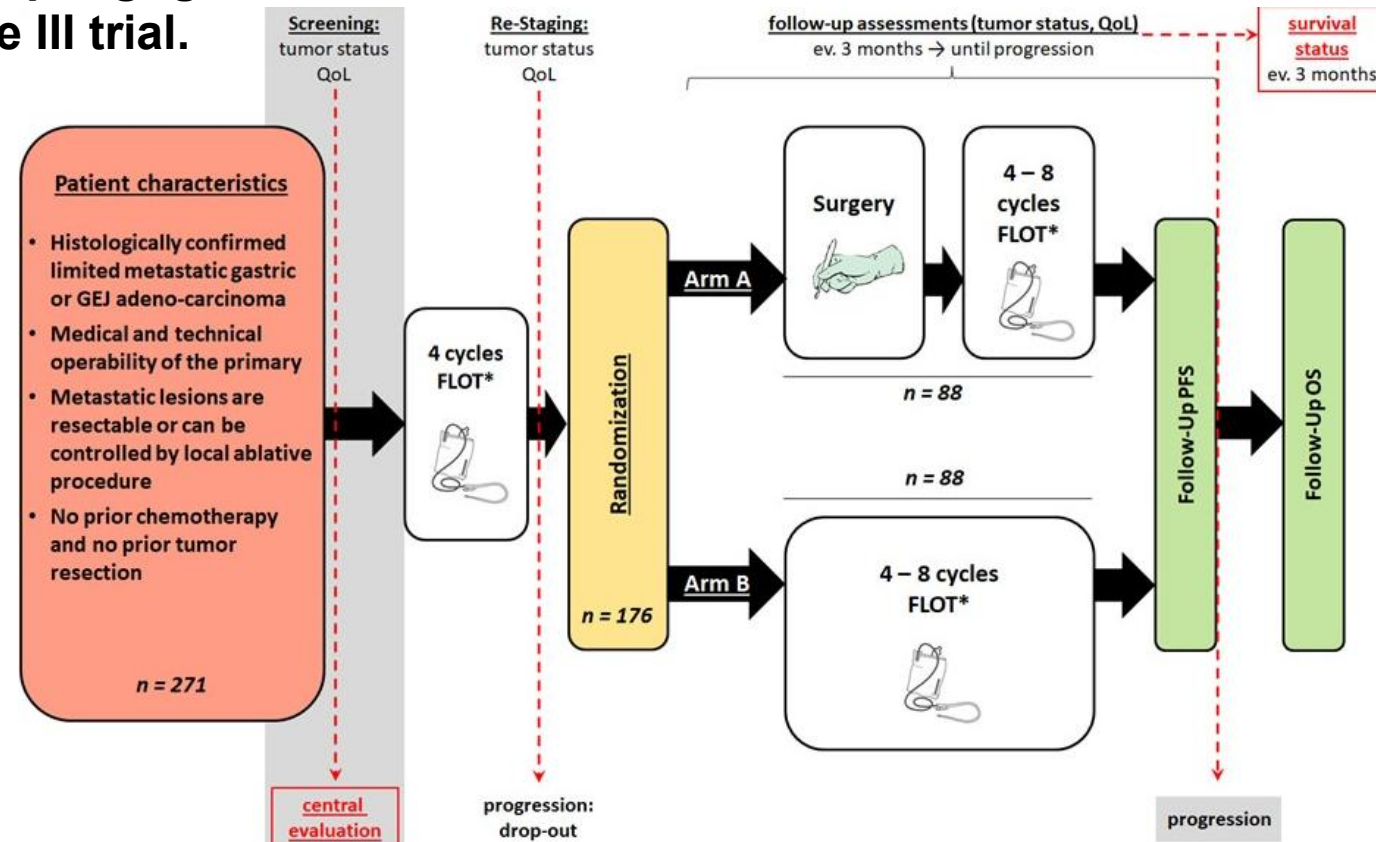
Resection of metastases cannot be recommended in general, but might be considered as an individual approach in highly selected cases with oligometastatic disease and response to ChT

Case report – surgery or no surgery

Effect of chemotherapy/targeted therapy alone vs. chemotherapy/targeted therapy followed by radical surgical resection on survival and quality of life in patients with limited-metastatic adenocarcinoma of the stomach or esophagogastric junction: The IKF-575/RENAISSANCE phase III trial.

Salah-Eddin Al-Batran, Sylvie Lorenzen, Jorge Riera, Karel Caca, Christian Mueller, Daniel E Stange, Thomas Zander, Claus Bolling, Nils Homann, Jochen Gaedcke, Peter C. Thuss-Patience, Patrick Michl, Wolfgang Blau, Kai Wille, Christine Koch, Claudia Pauligk, Daniel Wilhelm Mueller, Ulli Simone Bankstahl, Stefan Paul Mönig, Thorsten Goetze

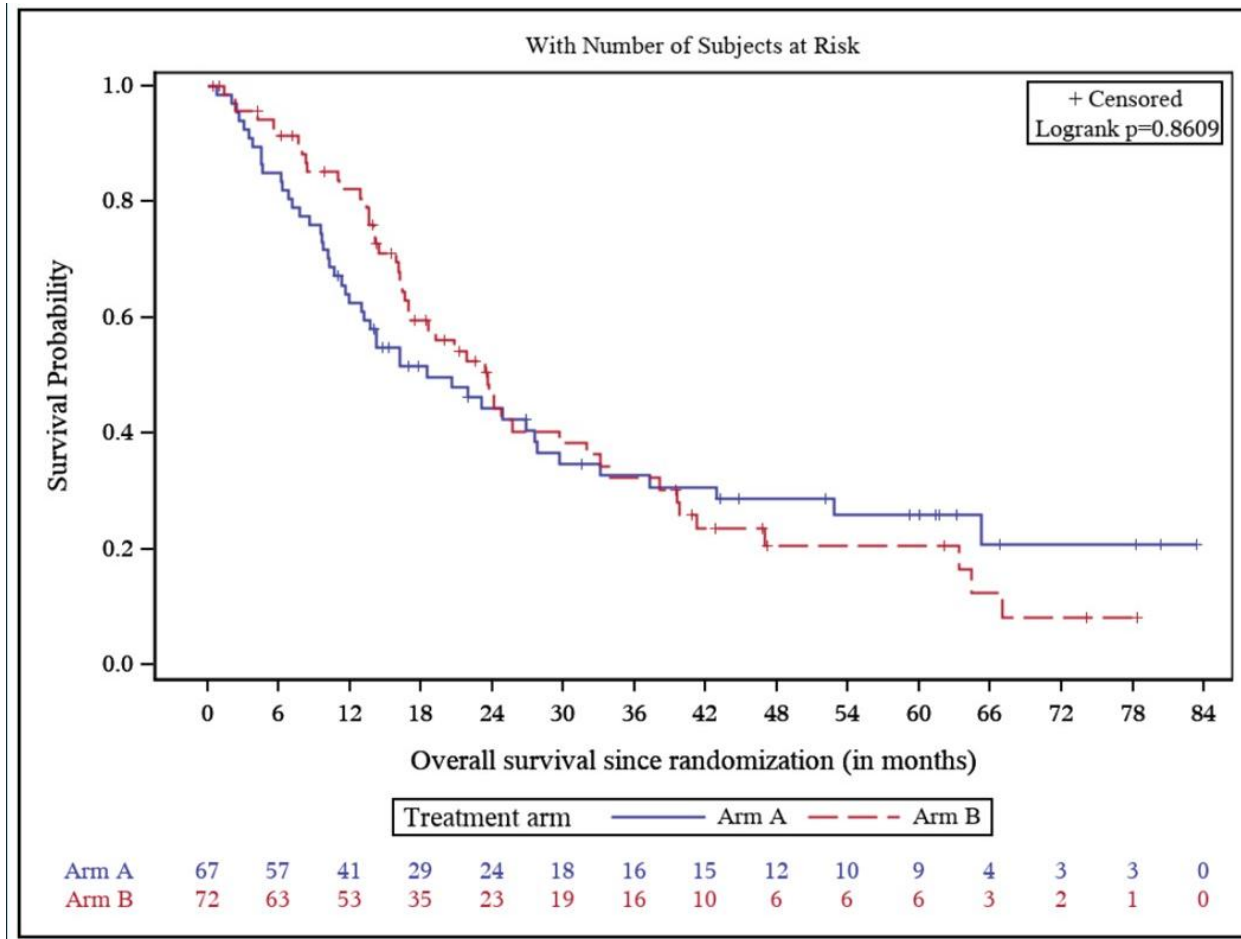
Presented at ASCO 2024



FLOT*: FLOT and if applicable combined with Trastuzumab or Nivolumab

IKF-575/RENAISSANCE

Overall survival – primary endpoint

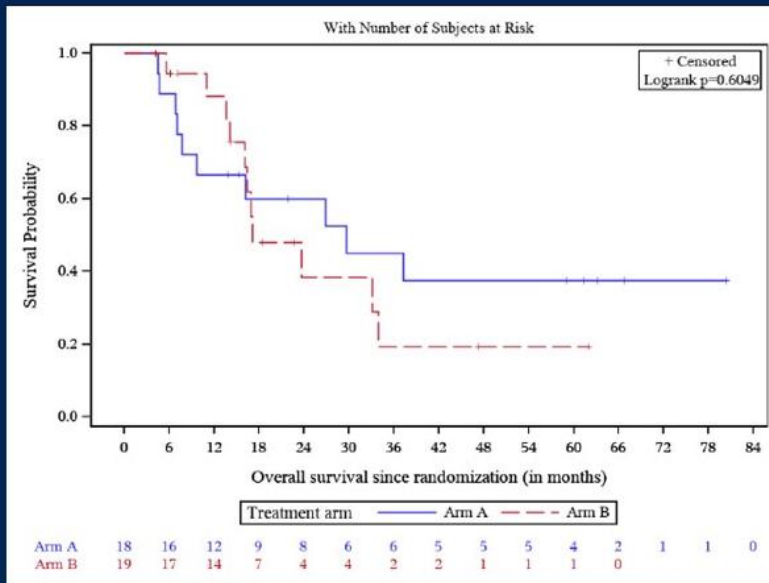


	Arm A (N= 67)	Arm B (N= 72)
Time until event, months [95% CI]		
25% Quantile	9.5 [4.6, 11.6]	14.0 [8.3, 16.4]
Median	18.5 [11.9, 27.7]	23.6 [16.6, 31.9]
75% Quantile	65.2 [27.7, -]	41.2 [31.9, 67.0]
Log Rank Test: p = 0.8609		
Cox Proportional Hazard Model: Hazard Ratio (95% CI), 1.037 (0.691 - 1.556), p = 0.8610		

IKF-575/RENAISSANCE

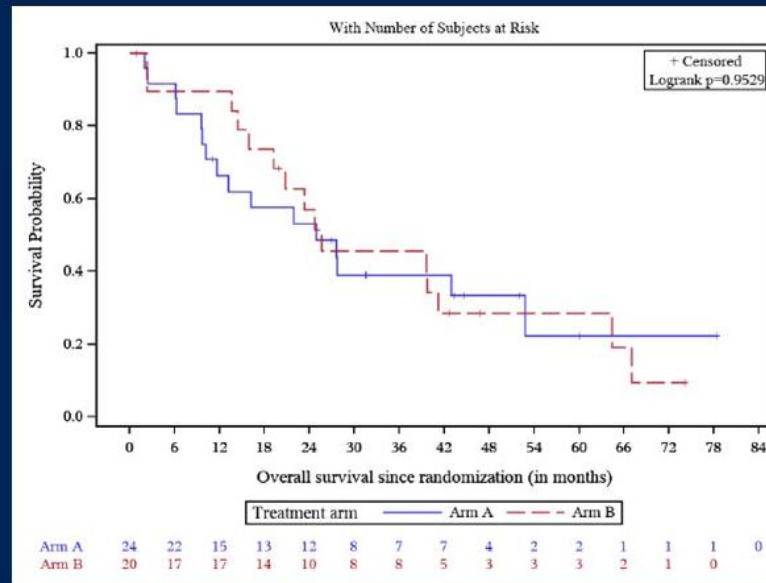
Overall survival – subgroup analysis

RPLN only (subgroup 1)



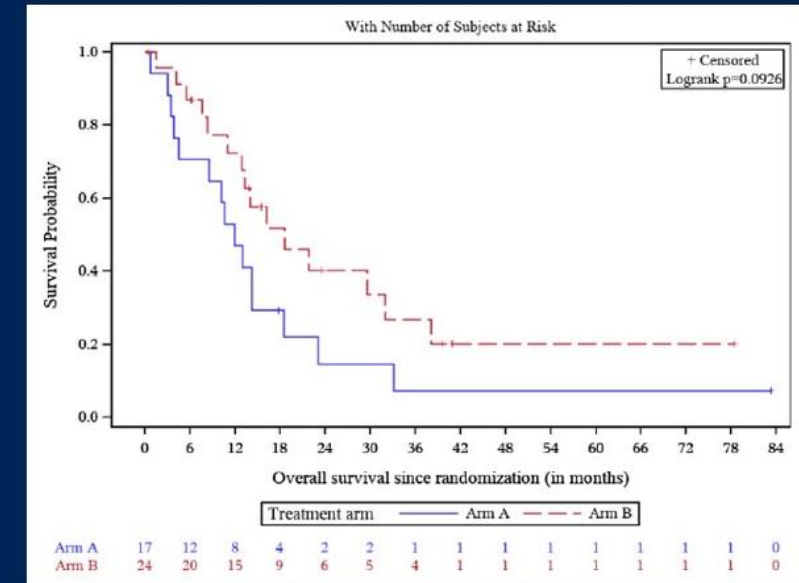
Median OS 29.6 vs. 17.1 months
36-mon OS 45% vs. 19%

Liver metastases (subgroup 2.II)



Median OS 24.9 vs. 25.7 months
36-mon OS 39% vs. 46%

Peritoneal metastases (subgroup 2.I)



Median OS 11.9 vs. 18.6 months
36-mon OS 7% vs. 27%

Case report – treatment cont.

ECOG PS 0, excellent condition, no tumor-related symptoms

Multidisciplinary team decided to offer the surgery for patient

The patient will undergo resection next week

Conclusions

Surgery is feasible only **in highly selected patient population**

Only **after several months (at least 4 months) of systemic treatment with response**

*Will the new **treatment modalities** (immunotherapy, anti-HER2, anti-Claudin), impact these recommendations?*

Surgery with **R0 resection of all tumor sites**

Highly experienced multidisciplinary team (especially surgical team)